

TRANSFORMING AMERICAN HEALTHCARE

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REFERENCES BY CHAPTER

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Federal Data & Reports

- Centers for Medicare & Medicaid Services. *National Health Expenditure Accounts: Historical*. Baltimore, MD: CMS; 2024. Available at: cms.gov/Research-Statistics-Data-and-Systems/Statistics-Trends-and-Reports/NationalHealthExpendData
- Commonwealth Fund. *Mirror, Mirror 2024: A Portrait of the Failing U.S. Health System*. New York: Commonwealth Fund; 2024. doi:10.26099/dxdy-5v16
- Anderson GF, Hussey P, Petrosyan V. It's Still the Prices, Stupid: Why the US Spends So Much on Health Care, and a Tribute to Uwe Reinhardt. *Health Affairs*. 2019;38(1):87–95. doi:10.1377/hlthaff.2018.05144
- National Academies of Sciences, Engineering, and Medicine. *The State of Health Disparities in the United States*. Washington, DC: National Academies Press; 2017.
- Papanicolas I, Woskie LR, Jha AK. Health Care Spending in the United States and Other High-Income Countries. *JAMA*. 2018;319(10):1024–1039. doi:10.1001/jama.2018.1150
- OECD. *Health at a Glance 2023: OECD Indicators*. Paris: OECD Publishing; 2023. doi:10.1787/7a7afb35-en
- Himmelstein DU, Campbell T, Woolhandler S. Health Care Administrative Costs in the United States and Canada, 2017. *New England Journal of Medicine*. 2020;382(15):1497–1498. doi:10.1056/NEJMc1917997
- McGinnis JM, Williams-Russo P, Knickman JR. The Case for More Active Policy Attention to Health Promotion. *Health Affairs*. 2002;21(2):78–93.

Chapter 2: The Six-Pillar Framework

Foundational Framework Documents

- Health Transformation Review. *White Paper — Health Transformation Review: A Six-Pillar Framework for U.S. Healthcare Transformation*. Version 4.7.0. Burlington, VT: HTR; April 2026.
- Berwick DM, Nolan TW, Whittington J. The Triple Aim: Care, Health, and Cost. *Health Affairs*. 2008;27(3):759–769. doi:10.1377/hlthaff.27.3.759
- Bodenheimer T, Sinsky C. From Triple to Quadruple Aim: Care of the Patient Requires Care of the Provider. *Annals of Family Medicine*. 2014;12(6):573–576.
- Stoto MA, Pardes H, eds. *For the Public's Health: Investing in a Healthier Future*. Washington, DC: National Academies Press; 2012.
- Porter ME, Lee TH. The Strategy That Will Fix Health Care. *Harvard Business Review*. October 2013. Available at: hbr.org

Chapter 3: The Health Transformation Index

Methodology Sources

14.

Health Transformation Review. *HTI Methodology — Health Transformation Index*. Version 4.7.0. Burlington, VT: HTR; April 2026.

15.

Health Care Payment Learning and Action Network (HCPLAN). *APM Measurement: Progress of the Healthcare System's Transition to Alternative Payment Models*. McLean, VA: HCPLAN; 2024.

16.

National Committee for Quality Assurance. *HEDIS & Performance Measurement*. Washington, DC: NCQA; 2025. Available at: ncqa.org/hedis

17.

CMS Innovation Center. *Innovation Models*. Baltimore, MD: CMS; 2025. Available at: innovation.cms.gov/innovation-models

18.

Office of the National Coordinator for Health Information Technology. *Health IT Dashboard*. Washington, DC: ONC; 2025. Available at: dashboard.healthit.gov

19.

Agency for Healthcare Research and Quality. *AHRQ Quality Indicators: Prevention Quality Indicators*. Rockville, MD: AHRQ; 2024. Available at: qualityindicators.ahrq.gov

20.

Kaiser Family Foundation. *State Health Facts*. San Francisco, CA: KFF; 2025. Available at: kff.org/statedata

Chapter 4: Policy

ACA and Federal Legislation

21.

Patient Protection and Affordable Care Act, Pub. L. No. 111-148, 124 Stat. 119 (2010).

22.

21st Century Cures Act, Pub. L. No. 114-255, 130 Stat. 1033 (2016).

23.

Inflation Reduction Act of 2022, Pub. L. No. 117-169, 136 Stat. 1818 (2022).

24.

No Surprises Act, Pub. L. No. 116-260, Division BB (2020).

25.

CMS. *CMS Interoperability and Prior Authorization Final Rule (CMS-0057-F)*. Baltimore, MD: CMS; January 2024.

CMMI Models

26.

McWilliams JM. CMMI at 10 Years. *New England Journal of Medicine*. 2021;384(8):765–769. doi:10.1056/NEJMp2033586

27.

Leavitt Partners. *CMMI Strategic Refresh: Implications for Value-Based Care*. Salt Lake City, UT: Leavitt Partners; 2022.

28.

CMS Innovation Center. *CMS Innovation Center Strategy Refresh*. Baltimore, MD: CMMI; October 2021. Available at: innovation.cms.gov

Prior Authorization

29.

American Medical Association. *2024 AMA Prior Authorization Physician Survey*. Chicago, IL: AMA; 2024. Available at: ama-assn.org/practice-management/prior-authorization/prior-authorization-data

30.

Heyward J, Bhatt A. Prior Authorization Reforms: Opportunities and Obstacles. *New England Journal of Medicine*. 2023;388(2):187–190.

State Policy

31.

Kaiser Family Foundation. *Status of State Medicaid Expansion Decisions: Interactive Map*. KFF; 2025. Available at: kff.org/medicaid/issue-brief/status-of-state-medicaid-expansion-decisions-interactive-map/

32.

National Academy for State Health Policy. *State Reinsurance Programs under the ACA: A Status Update*. Portland, ME: NASHP; 2024.

Chapter 5: Economics

Health Economics Foundational

33. Papanicolas I, Woskie LR, Jha AK. Health Care Spending in the United States and Other High-Income Countries. *JAMA*. 2018;319(10):1024–1039.

34. Getzen TE. *Health Economics and Financing*. 6th ed. Hoboken, NJ: John Wiley & Sons; 2018.

35. Folland S, Goodman AC, Stano M. *The Economics of Health and Health Care*. 8th ed. New York: Routledge; 2017.

Value-Based Care

36. Muhlestein D, Saunders R, Richards R, McClellan M. Recent Progress in the Value-Based Care Movement. *Health Affairs*. 2017;36(1):145–151.

37. McWilliams JM. Changes in Medicare Shared Savings Program Savings from 2013 to 2014. *JAMA*. 2016;316(16):1711–1713.

38. Song Z, Ji Y, Safran DG, Chernew ME. Health Care Spending, Utilization, and Quality 8 Years into Global Payment. *New England Journal of Medicine*. 2019;381(3):252–263.

Cost-Effectiveness Analysis

39. Neumann PJ, Sanders GD, Russell LB, Siegel JE, Ganiats TG, eds. *Cost-Effectiveness in Health and Medicine*. 2nd ed. Oxford: Oxford University Press; 2016.

40. Institute for Clinical and Economic Review. *ICER Value Assessment Framework for 2020–2023*. Boston, MA: ICER; 2020. Available at: icer.org/our-approach/methods-process/value-assessment-framework

41. Gold MR, Siegel JE, Russell LB, Weinstein MC, eds. *Cost-Effectiveness in Health and Medicine*. New York: Oxford University Press; 1996.

Private Equity

42. Bruch JD, Bellamy G, Bhatt AB. Assessment of Changes in Rural Hospital Services Following Private Equity Acquisition. *JAMA Internal Medicine*. 2023;183(7):747–754. doi:10.1001/jamainternmed.2023.1247

43. Amin K, Cox C, Bai G, Anderson GF. Private Equity In Health Care. *Health Affairs*. Health Policy Brief. June 2021.

Hospital Finance

44. American Hospital Association. *AHA Annual Survey of Hospitals*. Chicago, IL: AHA; 2024.

45. Healthcare Financial Management Association. *Revenue Cycle Benchmark Survey*. Westchester, IL: HFMA; 2025.

Chapter 6: Technology

FHIR and Interoperability

46. HL7 International. *HL7 FHIR Release 4*. Ann Arbor, MI: HL7; 2019. Available at: hl7.org/fhir/R4

47. US Core Implementation Guide. Version 7.0.0. Available at: hl7.org/fhir/us/core

48. Office of the National Coordinator for Health Information Technology. *ONC Interoperability and Patient Access Rule: Final Rule Overview*. Washington, DC: ONC; 2020.

49. Mandl KD, Kohane IS. A 21st-Century Health IT System. *New England Journal of Medicine*. 2017;376(6):507–509.

50. Holmgren AJ, Everson J, Adler-Milstein J. Hospital Adoption of Interoperability Functionality. *Health Affairs*. 2022;41(3):383–391.

Artificial Intelligence

51.

Obermeyer Z, Powers B, Vogeli C, Mullainathan S. Dissecting Racial Bias in an Algorithm Used to Manage the Health of Populations. *Science*. 2019;366(6464):447–453. doi:10.1126/science.aax2342

52.

Topol EJ. *Deep Medicine: How Artificial Intelligence Can Make Healthcare Human Again*. New York: Basic Books; 2019.

53.

Wiens J, Saria S, Sendak M, et al. Do No Harm: A Roadmap for Responsible Machine Learning for Health Care. *Nature Medicine*. 2019;25(9):1337–1340.

54.

FDA. *Artificial Intelligence/Machine Learning (AI/ML)-Based Software as a Medical Device (SaMD) Action Plan*. Silver Spring, MD: FDA; January 2021.

55.

Rajpurkar P, Chen E, Banerjee O, Topol EJ. AI in Health and Medicine. *Nature Medicine*. 2022;28(1):31–38.

Telehealth

56.

Koma W, Cubanski J, Neuman T. Telehealth Coverage and Reimbursement Policies in Medicare. *Health Affairs*. March 2024.

57.

Mehrotra A, Sherma L, Cai C, Luo J. Utilization of Telemedicine Among Rural Medicare Beneficiaries. *JAMA*. 2020;323(10):978–980.

Chapter 7: Clinical

Quality Measurement

58.

National Committee for Quality Assurance. *HEDIS Technical Specifications for Health Plans*. Washington, DC: NCQA; 2025.

59.

CMS. *Medicare Advantage Star Ratings Technical Notes*. Baltimore, MD: CMS; 2024.

60.

CMS. *Quality Payment Program: MIPS Overview*. Baltimore, MD: CMS; 2025. Available at: qpp.cms.gov

Risk Stratification

61.

Pope GC, Kautter J, Ingber MJ, et al. Evaluation of the CMS-HCC Risk Adjustment Model. *CMS Research Report*. Baltimore, MD: CMS; 2011.

62.

Elixhauser A, Steiner C, Harris DR, Coffey RM. Comorbidity Measures for Use with Administrative Data. *Medical Care*. 1998;36(1):8–27.

63.

Charlson ME, Pompei P, Ales KL, MacKenzie CR. A New Method of Classifying Prognostic Comorbidity in Longitudinal Studies: Development and Validation. *Journal of Chronic Diseases*. 1987;40(5):373–383.

Care Delivery Models

64.

Patient-Centered Primary Care Collaborative. *Defining the Medical Home*. Washington, DC: PCPCC; 2024. Available at: pcpcc.org

65.

Fisher ES, Staiger DO, Bynum JP, Gottlieb DJ. Creating Accountable Care Organizations: The Extended Hospital Medical Staff. *Health Affairs*. 2007;26(1):w44–w57.

66.

Unützer J, Katon W, Callahan CM, et al. Collaborative Care Management of Late-Life Depression in the Primary Care Setting: A Randomized Controlled Trial. *JAMA*. 2002;288(22):2836–2845.

Workforce

67.

Health Resources and Services Administration. *National and Regional Projections of Supply and Demand for Primary Care Practitioners: 2013–2025*. Rockville, MD: HRSA; 2016.

68.

American Association of Colleges of Nursing. *Nursing Faculty Shortage Fact Sheet*. Washington, DC: AACN; 2024.

69.

Dill MJ, Salsberg ES. *The Complexities of Physician Supply and Demand*. Washington, DC: Association of American Medical Colleges; 2008.

Chapter 8: Health Equity

Disparities Research

70. Agency for Healthcare Research and Quality. *National Healthcare Quality and Disparities Report 2023*. Rockville, MD: AHRQ; 2023. AHRQ Pub. No. 24-0005.

71. Williams DR, Rucker TD. Understanding and Addressing Racial Disparities in Health Care. *Health Care Financing Review*. 2000;21(4):75–90.

72. Artiga S, Foutz J, Cornachione E, Garfield R. *Key Facts on Health and Health Care by Race and Ethnicity*. San Francisco, CA: KFF; 2024.

73. Rural Health Research Gateway. *Rural Health Disparities*. University of North Dakota Center for Rural Health; 2024.

Social Determinants of Health

74. Marmot M, Wilkinson RG, eds. *Social Determinants of Health*. 2nd ed. Oxford: Oxford University Press; 2006.

75. Braveman P, Gottlieb L. The Social Determinants of Health: It's Time to Consider the Causes of the Causes. *Public Health Reports*. 2014;129(Suppl 2):19–31.

76. Gottlieb L, Cottrell EK, Park B, Clark KD, Gold R, Fichtenberg C. Advancing Social Prescribing with Implementation Science. *Journal of the American Board of Family Medicine*. 2018;31(3):294–299.

77. Berkowitz SA, Hulberg AC, Standish S, Reznor G, Atlas SJ. Addressing Unmet Basic Resource Needs as Part of Chronic Cardiometabolic Disease Management. *JAMA Internal Medicine*. 2017;177(2):244–252.

Equity Measurement and HEROI

78. Gidwani R, Soto-Perez-de-Celis E. Equity in Cost-Effectiveness Analyses. *New England Journal of Medicine*. 2020;382(11):999–1001.

79. Cookson R, Mirelman AJ, Griffin S, et al. Using Cost-Effectiveness Analysis to Address Health Equity Concerns. *Value in Health*. 2017;20(2):206–212.

80. National Committee for Quality Assurance. *Health Equity Accreditation Program*. Washington, DC: NCQA; 2024. Available at: ncqa.org/programs/health-equity

Chapter 9: Operations

Revenue Cycle

81. Healthcare Financial Management Association. *MAP Keys: Best Practices in Revenue Cycle*. Westchester, IL: HFMA; 2025.

82. Advisory Board. *Revenue Cycle Benchmarking Database*. Washington, DC: Advisory Board; 2024.

83. Tseng P, Kaplan RS, Richman BD, Shah MA, Schulman KA. Administrative Costs Associated with Physician Billing and Insurance-Related Activities at an Academic Health Care System. *JAMA*. 2018;319(7):691–697.

Administrative Costs

84. Himmelstein DU, Woolhandler S. The Current and Projected Taxpayer Shares of US Health Costs. *American Journal of Public Health*. 2016;106(3):449–452.

85. Cutler DM. *The Quality Cure: How Focusing on Health Care Quality Can Save Your Life and Lower Spending Too*. Berkeley: University of California Press; 2014.

Workforce Operations

86. NSI Nursing Solutions Inc. *2024 NSI National Health Care Retention & RN Staffing Report*. Warwick, RI: NSI; 2024.

87. Pfeffer J, Veiga JF. Putting People First for Organizational Success. *Academy of Management Executive*. 1999;13(2):37–48.

Chapters 10–12: Vermont

Primary Sources

88. Oliver Wyman/Marsh McLennan. *Vermont Act 167 Community Engagement: Recommendations*. Boston, MA: Oliver Wyman; August 2024.

89. Health Transformation Review. *Vermont Act 167 — HTR Advisory Analysis*. Burlington, VT: HTR; March 2026.

90. Vermont Act 167, H.861. General Assembly of the State of Vermont; 2024.

91. Vermont Act 48, H.202 (Green Mountain Care). General Assembly of the State of Vermont; 2011.

AHEAD Model

92. Center for Medicare and Medicaid Innovation. *All-Payer Health Equity Approaches and Development (AHEAD) Model*. Baltimore, MD: CMMI; 2022. Available at: innovation.cms.gov/innovation-models/ahead

93. Green Mountain Care Board. *AHEAD Model Annual Report*. Montpelier, VT: GMCB; 2025.

94. Department of Vermont Health Access. *Vermont Medicaid Annual Report*. Waterbury, VT: DVHA; 2025.

Vermont Healthcare Data

95. Vermont Department of Financial Regulation. *Vermont Health Insurance Market Report*. Montpelier, VT: DFR; 2025.

96. Vermont Health Care Uniform Reporting and Evaluation System (VHCURES). *Vermont All-Payer Claims Data Summary*. Montpelier, VT: GMCB; 2024.

97. Rural Health Research Gateway. *Vermont Rural Hospital Financial Status*. University of North Dakota; 2024.

Global Budget Evidence Base

98. Berkowitz SA, Ishii L, Schuler S, et al. Maryland's All-Payer Model—Results After 6 Years. *New England Journal of Medicine*. 2022;387(22):2021–2023.

99. Kocher R, Sahni NR. Hospitals' Race to Employ Physicians — The Logic Behind a Money-Losing Proposition. *New England Journal of Medicine*. 2011;364(19):1790–1793.

100. Song Z, Rose S, Safran DG, Landon BE, Day MP, Chernew ME. Changes in Health Care Spending and Quality 4 Years into Global Payment. *New England Journal of Medicine*. 2014;371(18):1704–1714.

Chapters 13–15: Tools, Academy, AI

Research Methods

101. Briggs AH, Claxton K, Sculpher M. *Decision Modelling for Health Economic Evaluation*. Oxford: Oxford University Press; 2006.

102. Siebert U, Alagoz O, Bayoumi AM, et al. State-Transition Modeling: A Report of the ISPOR-SMDM Modeling Good Research Practices Task Force. *Value in Health*. 2012;15(6):812–820.

103. Kermack WO, McKendrick AG. A Contribution to the Mathematical Theory of Epidemics. *Proceedings of the Royal Society of London Series A*. 1927;115(772):700–721.

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104.

Lewis P, Perez E, Piktus A, et al. Retrieval-Augmented Generation for Knowledge-Intensive NLP Tasks. *Advances in Neural Information Processing Systems*. 2020;33:9459–9474.

105.

Gao Y, Xiong Y, Gao X, et al. Retrieval-Augmented Generation for Large Language Models: A Survey. *arXiv preprint*. arXiv:2312.10997; 2024.

106.

Robertson S, Walker S. Some Simple Effective Approximations to the 2-Poisson Model for Probabilistic Weighted Retrieval. *SIGIR '94: Proceedings of the 17th Annual International ACM SIGIR Conference*. 1994:232–241.

107.

Nogueira R, Jiang Z, Lin J. Document Ranking with a Pretrained Sequence-to-Sequence Model. *arXiv preprint*. arXiv:2003.06713; 2020.

Learning Science

108.

Ambrose SA, Bridges MW, DiPietro M, Lovett MC, Norman MK. *How Learning Works: Seven Research-Based Principles for Smart Teaching*. San Francisco: Jossey-Bass; 2010.

109.

Knowles MS, Holton EF, Swanson RA. *The Adult Learner: The Definitive Classic in Adult Education and Human Resource Development*. 9th ed. New York: Routledge; 2020.

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110.

Porter ME. *Competitive Strategy: Techniques for Analyzing Industries and Competitors*. New York: Free Press; 1980.

111.

Kaplan RS, Anderson SR. *Time-Driven Activity-Based Costing*. Boston: Harvard Business School Press; 2007.

Healthcare Futures

112.

Emanuel EJ, Gudbranson E, Van Parys J, Gándara M, Lim J, Levy J. Comparing Health Outcomes of Privileged US Citizens with Those of Average Residents of Other Developed Nations. *JAMA Internal Medicine*. 2021;181(3):339–344.

113.

Chernew ME, McWilliams JM, Landon BE. Value-Based Insurance Design: The Implications for Physicians. *JAMA*. 2011;305(2):185–186.

114.

Blumenthal D, Fowler EJ, Abrams M, Collins SR. Covid-19 — Implications for the Health Care System. *New England Journal of Medicine*. 2020;383(15):1483–1488.

115.

Health Transformation Review. *Platform Improvement Plan 2025–2027*. Version 4.7.0. Burlington, VT: HTR; 2026.

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